PROCEDURE: Intravitreal injection of aflibercept (Eylea 2.0 mg / 0.05 mL), left eye.

INDICATION: Active neovascular (wet) age-related macular degeneration (nAMD) with persistent subretinal and intraretinal fluid documented on optical coherence tomography (OCT).

PROCEDURE NOTE: Informed consent was obtained with discussion of risks including endophthalmitis, retinal detachment, vitreous hemorrhage, and IOP elevation. The patient was positioned supine in the examination chair. Topical proparacaine 0.5% drops were instilled into the left eye every 2 minutes for 3 cycles.

A sterile eyelid speculum was placed. Povidone-iodine (Betadine) 5% solution was instilled onto the ocular surface, fornices, and eyelid margins and allowed to dwell for 3 minutes to ensure surgical antisepsis. A sterile cotton-tipped applicator soaked in 4% lidocaine was held firmly against the inferotemporal conjunctiva for 60 seconds.

Using surgical calipers, an injection site was marked 3.5 mm posterior to the limbus in the inferotemporal quadrant (pseudophakic patient). A 30-gauge 1/2-inch needle attached to a tuberculin syringe containing 0.05 mL of aflibercept was introduced smoothly through the pars plana into the mid-vitreous cavity with the needle tip aimed toward the optic nerve. The medication was slowly injected, and the needle withdrawn while applying a sterile cotton-tipped applicator over the entry site to prevent reflux.

Gross visual acuity was verified as count fingers immediately post-injection. Indirect ophthalmoscopy confirmed central retinal artery perfusion and absence of retinal tears. Betadine was thoroughly irrigated with sterile saline. The patient was provided post-injection emergency instructions and scheduled for follow-up OCT in 4 weeks.